

EXERCISES AND EXACT SOLUTIONS

V7_B_N15 - National Health Intelligence Capstone

Independent practice, facilitated discussion, and assessment reference

Production Unit	PU-B07-C35
Notebook	V7_B_N15
Version	0.1
Assessment boundary	Learning evidence—not operational authorization

Instructions

Complete Part A before consulting the solutions. State assumptions explicitly. Where evidence is insufficient, write an abstention or escalation response rather than inventing precision.

Exercise 1

Separate surveillance from diagnosis.

- Required output: a concise decision-ready response with owner, evidence, uncertainty, and boundary where applicable.

Exercise 2

Design an incident handoff.

- Required output: a concise decision-ready response with owner, evidence, uncertainty, and boundary where applicable.

Exercise 3

Audit equity under scarce capacity.

- Required output: a concise decision-ready response with owner, evidence, uncertainty, and boundary where applicable.

Exact solutions and scoring guide

Solution 1

Surveillance detects population signals for verification; diagnosis is a licensed individual clinical act.

Solution 2

Record evidence, uncertainty, urgency, owner, timestamp, command acceptance, action, override, and closure review.

Solution 3

Examine need, access barriers, false errors, waiting time, benefit, harm, and appeal across groups without ranking human worth.

Scoring rubric

Criterion	Strong evidence	Revision required
Decision framing	Owner, action, horizon, authority named	Model output substituted for decision
Evidence	Concepts, grain, provenance, limits explicit	Data treated as self-explanatory
Uncertainty	Abstention/escalation used appropriately	False precision or hidden limitations
Governance	Human authority and safeguards retained	Authority silently delegated